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Positioning a Pharmacokinetic Instrument in a Hostile and Under-Served Market:

A Market, Cultural, and Nomenclature Study of xT, with Screened Rebrand Candidates

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Abstract

xT is a mobile application that predicts serum hormone concentration over time from a user's logged dose history for gender-affirming hormone replacement therapy (HRT), built on a superposed Bateman biexponential model with per-user calibration. Its engineering is documented to a standard unusual for a consumer application. Its name is a development codename. This paper asks what market the product is actually entering, what that market's cultural and political conditions demand of a brand, and what the application should be called.

We find three things that revise the project's own prior positioning statements. First, the trans-specific segment of this category is not competitive but *vacant*: its most-cited entrants—Transcapsule, Solace/Euphoria, MyTransHealth—are abandoned, with last updates in 2022, 2022, and 2019 respectively, while the actively maintained, well-rated products in the adjacent space are coded to cisgender male optimization (Regimen, 338 ratings, 4.8, updated within the week) or GLP-1 weight loss (DoneDose). The competitive threat is not feature parity; it is that this category has taught its users that tools built for them get abandoned. Second, and correcting an explicit claim in the project's own business plan, the pharmacokinetic model is *not* a unique differentiator: Regimen ships a public one-compartment Bateman superposition visualizer with cited sources and stated limitations. xT's defensible asset is not that it models pharmacokinetics but the depth and honesty of its disclosure—a published derivation, an out-of-sample validation dossier, and a revision history that names its own defects. Third, the brand brief the founder has set—clinically credible *and* discreet—is not a stylistic preference but a safety requirement, and it is the specific intersection at which every prior entrant in this category has failed.

We evaluate rebrand candidates against a four-part discretion test (lock screen, financial record, over-the-shoulder, and disclosure under compulsion) and a clearance screen covering App Store collisions and United States trademark records in classes 9, 42, and 44. We report seven screened candidates and eleven rejections with reasons, and we benchmark the incumbent codename against them. We recommend Isocline, with Ephemeris as the principal alternate and a defensible case for retaining xT stated in full rather than strawmanned.

Clearance findings in this paper are indicative, not legal opinions. Where a source is a competitor's own marketing, we say so.

Keywords: market positioning, brand nomenclature, trademark screening, gender-affirming care, digital health, app store optimization, health data privacy, trans health technology

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1 Introduction

1.1 The problem this paper addresses

xT began as a directory name. It is short, it gestures at testosterone, it is at least ungendered in the sense that it does not commit the product to one hormone—but nobody chose it, and no part of it was tested against the conditions the product will actually ship into. The question posed for this study was whether xT should be renamed, and if so, to what.

That question cannot be answered by generating names. A name is a solution to a positioning problem, and the positioning problem here is unusual in three ways that a generic naming exercise would miss:

1. The product's technical asset is a pharmacokinetic model of unusual rigor for a consumer application, which means the brand has to be able to carry a scientific claim without over-claiming a regulatory one.
2. The user population is, in a material and documented sense, under surveillance. A name is a string that appears on a lock screen, a bank statement, and a subpoenaed app-store purchase record. In this category that is a safety property, not a marketing one.
3. The category has a mortality problem. The applications this product would be compared against are, disproportionately, dead. That fact shapes what a new entrant's brand has to communicate more than any feature comparison does.

1.2 Scope

The founder scoped this study to three of four candidate research areas: the competitive application landscape (§3), the social, cultural, and political environment (§4), and brand and category positioning (§5). Medical and clinical positioning —provider adoption, WPATH and Endocrine Society context, HIPAA posture, and the software-as-a-medical-device question—was explicitly deprioritized and is not covered here.

One exception is taken to that exclusion, and it is flagged rather than smuggled. In March 2026 Apple began requiring apps in the Health & Fitness and Medical categories to declare a regulated-medical-device status, with an early-2027 deadline after which undeclared apps cannot ship updates [2]. This is a regulatory fact, but it constrains what the brand is permitted to *claim*, and therefore falls inside the positioning scope rather than outside it. It is treated in §5.4 and nowhere else.

1.3 The brand brief

The founder specified two optimization targets, selected together:

Clinically credible.
The name should read as a serious health instrument—provider-shareable, grant-fundable, and able to sit on the title page of a research paper without embarrassment.

Discreet and ungendered.
The name should disclose nothing on a lock screen or a financial record.

These are not independent axes, and the fact that they were selected together is the most consequential input to this study. Section 5.1 shows that the intersection of the two is a narrow region that no existing product in this category occupies, that the occupied regions are occupied for legible reasons, and that most of the obvious naming instincts—including several this study generated and then discarded—fail one axis in the course of satisfying the other.

1.4 Contributions

1. A competitive census of the HRT-tracking category built from primary App Store records rather than from competitor-authored comparison articles (§3.2), including last-updated dates, rating counts, and pricing.
2. An abandonment analysis showing that the trans-specific segment is vacant rather than contested, and an argument that this changes what the brand must do (§3.3).
3. A correction to the project's own business plan: the claim that no competitor models pharmacokinetics is false (§3.4).
4. A four-part discretion test operationalizing the "discreet" half of the brief (§6.1).
5. Seven screened rebrand candidates and eleven documented rejections, with App Store and trademark screens (§6.3, §6.4).
6. A stated case for retaining the incumbent codename, benchmarked against the candidates rather than dismissed (§6.3).

2 Method and source quality

2.1 Why this section exists

The pharmacokinetic paper accompanying this project grades its own constants—primary-sourced, secondary-sourced, or uncalibrated—and declines to present a secondary source as though it were a primary one. Market research is more prone to the same failure, not less, because its most available sources are written by interested parties. This paper adopts the same hierarchy.

2.2 Source tiers

Tier	Definition	Used in this paper for
Primary	Record produced by the party with direct knowledge, published for a purpose other than persuading the reader of a market conclusion.	App Store listing pages (pricing, rating counts, last-updated dates, seller names, declared data practices); USPTO records via Justia; Apple developer documentation and policy announcements.
Secondary	Independent third-party reporting or research with no commercial interest in the category.	Peer-reviewed and press reporting on DIY HRT prevalence; the Movement Advancement Project policy maps; POGO's surveillance investigation; appellate reporting.
Interested	Written by a market participant about its own market.	The "best HRT tracker apps" comparison articles published by Regimen and DoneDose. Used only for feature enumeration, never for ranking or evaluation.

Tier	Definition	Used in this paper for
Unverified	Asserted in a project document without a traceable source, or resistant to verification with the tools available.	Domain availability; the completeness of any trademark screen; several claims in the project's own business plan, one of which is corrected in §3.4.

2.3 A note on the comparison articles

Two of the most prominent search results for "best HRT tracker apps" are `helloregimen.com/blog/best-hrt-tracker-apps-2026` and `donedose.com/guides/best-hrt-tracker-app`. Both are content marketing published by applications that appear in their own rankings [11, 4]. Regimen's article places Regimen first. DoneDose's article places DoneDose first. Neither discloses the conflict in the body text.

This is not a scandal; it is the normal structure of search-optimized category content, and both articles are factually useful for enumerating who exists. But it has a specific consequence for this project: the competitive picture in the project's existing business plan is built substantially on these two articles, and therefore inherits their framing. Where this paper disagrees with that plan, the disagreement usually traces to having gone to the App Store listing instead.

2.4 What this paper cannot establish

Three limits are stated here rather than buried in §7, because they bear on how the rebrand section should be read.

Trademark screens here are not clearance opinions. The screens in §6.3 were run against Justia's index of United States federal trademark records, filtered to International Classes 9 (software), 42 (technology services), and 44 (medical services). They do not cover state registrations, unregistered common-law marks, foreign registrations, or the likelihood-of-confusion analysis that determines whether a mark is actually available. A mark reported here as "clear" means only that this screen surfaced no conflict in those classes. Retaining a trademark attorney before filing or before printing anything is a separate and necessary step.

Domain availability was not established. The tooling available for this study could not query a registrar. Domain notes in §6.3 are inferences from whether a site appears to exist, and should be re-checked directly—a task of a few minutes per name.

App Store name availability was not established. Whether a name can be reserved is determined in App Store Connect, not by searching the storefront. The App Store screens here establish only whether a user searching that word would encounter competing results.

3 The competitive landscape

3.1 Three overlapping markets, not one

"HRT tracking" names three populations that share a mechanism and share almost nothing else. Conflating them is the single most common error in this category's own marketing, and it is why the comparison articles read as incoherent—they rank a menopause education platform against an injection logger against a peptide tracker.

Market	Who	Emotional register	Representative apps
Menopausal HRT	Cisgender women, typically 45–60, usually clinician-managed, often national-health-service adjacent.	Symptom relief; being taken seriously.	Balance, Health & Her, Caria, mySysters, Hormona, HRTMe
Optimization / TRT / peptides	Predominantly cisgender men, self-directed, protocol-tinkering, overlapping with GLP-1 and bodybuilding communities.	Performance; control; data maximalism.	Regimen, DoneDose, TRT Tracker
Gender-affirming HRT	Trans and gender-diverse adults, frequently self-advocating, sometimes unsupervised, materially at legal and social risk.	Safety; self-determination; not being surveilled.	Transcapsule, Solace, Hormone Helper, Trans Memo

The three markets differ in what a product must earn. The menopause market rewards authority—Balance leads with its founder's clinical credentials and its alignment with national guidance [11]. The optimization market rewards granularity and rewards it immediately; Regimen's pitch is 200-plus compounds and 50-plus daily markers [13]. The gender-affirming market rewards neither of those first. It rewards evidence that the builder is not going to disappear, extract, or expose them—and it has extensive evidence that builders do all three.

3.2 Census of the actively relevant field

The following is built from App Store listing pages, which are primary records, and not from the comparison articles. Figures were retrieved in August 2026 and will drift.

App	Seller	Rating (n)	Price	Notes
Regimen: Peptide Tracker	Awaken Labs LLC	4.8 (338)	Free; $4.99/mo, $39.99/yr	Health & Fitness. Updated within the week of retrieval (v2.1.2). 200+ compounds, half-life plotting, labs, 50+ symptom markers, Apple Health. Health data linked to user ID.

App	Seller	Rating (n)	Price	Notes
DoneDose: Injection Tracker	Benjamin Fox	4.0 (5)	Free; $4.99/mo, $34.99/yr, $4.99 lifetime	Health & Fitness. Latest version 1.1.2 dated "Jun 10" with no year shown—Apple omits the year for the current year, and the app ID is a 2025-series listing, so this reads as June 2026 and the app is actively maintained. GLP-1-first framing; TRT/HRT listed among many. Local storage, no cloud sync.
Hormone Helper	John Patterson	5.0 (7)	$4.99 paid	**Lifestyle** category, not Health. Last updated September 2024. Explicitly trans and non-binary framing; educational video content.
Transcapsule	Transcapsule	—	Free w/ premium	**Abandoned since 2022** per independent review [14]. Crowdfunded 2020. Promised anonymized research data sharing.
Solace / Euphoria	Euphoria.LGBT	—	Free	**Last updated 2022** [14]. Venture-backed. See §4.5.
MyTransHealth	—	—	—	**Dead. Last update 2019; site non-functional** [14].
Trans Memo	—	—	—	Actively maintained as of April 2026 [14]. HRT reminder and injection tracking. Lightweight.

Three observations follow directly from the table and are worth stating separately because they are easy to read past.

The ratings distribution is extreme. Regimen has 338 ratings. Every trans-specific product in the census has single digits or no retrievable count. This is not evidence that trans users do not want software; it is evidence that they have not been offered a maintained product to rate.

Hormone Helper is in the Lifestyle category. Its listing is categorized under Lifestyle rather than Health & Fitness or Medical [6]. Whether that was a positioning choice or an attempt to avoid health-app review scrutiny, the effect is that the most explicitly trans-framed maintained product in the census is not present in the category where users search for health tools. It is also, incidentally, the categorization that would exempt it from Apple's 2027 medical-device declaration requirement (§5.4).

DoneDose stores data locally with no cloud sync. This is the strongest privacy posture in the census and it belongs to a GLP-1-oriented product with five ratings. The feature that

the trans market would most reward is currently shipping in the product least aimed at it.

3.3 The abandonment pattern

The trans-specific segment of this category is not a competitive field. It is a graveyard, and it is documented as one by the community itself. A 2026 community-authored guide to applications for trans people characterizes Transcapsule as "abandoned" since 2022 and notes that its website auto-downloads software; MyTransHealth as dead with a non-functional site since 2019; and Solace as last updated in 2022 with outdated Android compatibility [14]. Of the sixteen applications that guide reviews, the ones it recommends without reservation for trans users are, overwhelmingly, not trans-specific products at all—Reddit, Finch, Google Maps' location sharing.

This is the central competitive finding of this study, and it inverts the usual strategic conclusion. The instinct on discovering an empty segment is to celebrate the opening. The correct reading is more uncomfortable: the segment is empty because entrants keep dying in it, and every prospective user of xT has watched that happen at least once.

The practical consequence is that the strongest competitor xT faces is not a feature set. It is a prior—a learned, evidence-backed expectation that a trans health app will be built by someone well-meaning, will be charming for eighteen months, and will then stop receiving updates while continuing to hold its users' hormone histories on a server nobody is watching. No feature answers that prior. What answers it is a name, an entity, and a publication record that together look like something built to persist.

That reframing has direct nomenclature consequences, developed in §5.2: it argues for a name with institutional weight over a name with warmth, because the thing being communicated is durability rather than empathy.

3.4 Pharmacokinetic modeling is not a moat

The project's existing business plan states that xT is "the only one (as far as this research found) that's publishing its own PK research memos and running empirical persona validation against the model," and that "nobody else in this space appears to be citing population-PK papers in their source comments."

The first clause is defensible. The second is not, and the broader implication—that competitors do not do pharmacokinetics—is false.

Regimen publishes a public Half-Life Visualizer that, by its own methodology statements, uses "the Bateman equation, a one-compartment pharmacokinetic model," applies superposition to layer multiple doses and show steady-state accumulation, and cites FDA NDA labels, a New England Journal of Medicine trial for retatrutide, and published literature for testosterone [12]. It carries limitation language that would not be out of place in xT's own documentation: "Population-average pharmacokinetic model. Individual variation is significant"; "real blood levels vary based on individual metabolism, body composition, injection site, formulation"; and an acknowledgement that the tool offers "a useful approximation" rather than clinical precision.

That is, to a first approximation, the same model class xT implements, with the same structural caveats, from a competitor with 338 ratings and a weekly release cadence.

This correction matters because a positioning built on a false differentiator fails on contact with a comparison. If the pitch is "we model pharmacokinetics," a reader who has seen Regimen's visualizer has already discounted it. The differentiators that survive the comparison are narrower and, fortunately, real:

1. **Derivation rather than assertion.** Regimen states that it uses a Bateman model. xT publishes the closed-form derivation of its scaling constant from a named population-PK

source, and a conservation-of-mass identity for the esters where no primary source exists. That is a different epistemic object.

2. **Published out-of-sample validation.** A validation dossier and a persona study reporting error bands against published anchors is not something any competitor in this census offers.
3. **Disclosed defects.** The current revision of xT's PK paper reports three defects found by re-deriving the model from its sources—including a baseline keyed to the wrong gonads, which over-states a transmasculine user's endogenous testosterone by more than an order of magnitude at treatment onset—and states their magnitudes rather than quietly repairing them. No competitor does this. In a market whose defining problem is earned distrust (§3.3), a published defect list is worth more than a feature.
4. **Trans-specific parameterization.** Regimen's compound library is organized around peptides, GLP-1 agonists, and TRT. Estradiol valerate and cypionate, anti-androgen interactions, and transfeminine dosing are, at best, long-tail entries in a product whose center of gravity is elsewhere.

The honest one-line claim is therefore not "we have a PK model." It is closer to: *the only hormone estimator that shows its work, and the only one that tells you where it is wrong.*

3.5 What the competitive set implies for the name

Two conventions are visible in the census and both constrain the brand.

First, the App Store naming convention in this category is *brand plus descriptor*: "Regimen: Peptide Tracker," "DoneDose: Injection Tracker." DoneDose in fact appears under at least two descriptor variants against a single app ID [5], which is ordinary keyword experimentation. Apple allows 30 characters of app name plus a 30-character subtitle [8]. This is strategically useful: the descriptor half can carry the search keywords, which frees the brand half to be abstract. A brand does not have to say "hormone" for the listing to rank for "hormone tracker."

Second, and consequently, the argument that an abstract name is "expensive to market" because it has to be taught—an argument made in the project's own monetization document against the byTime candidate—is weaker than it appears in a store that gives you a subtitle. The real cost of an abstract name is not comprehension. It is brand-term search competition, which is a function of collisions, not of abstractness. That distinction drives the screening in §6.3.

4 The social, cultural, and political environment

4.1 Why a naming study contains a legal section

In most categories, the political environment is a marketing context. Here it is a threat model. The argument of this section is that the "discreet" half of the brand brief is downstream of documented enforcement mechanisms, and that treating it as an aesthetic preference would be a category error.

4.2 Legal climate as of August 2026

Two facts establish the direction of travel.

Restrictions have moved from minors to adults, and a federal appellate court has now permitted it. On 11 March 2026 the United States Court of Appeals for the Fourth Circuit upheld West Virginia's exclusion of gender-affirming surgeries for adults from Medicaid coverage,

reasoning that because "the law applies to specific procedures, not specific individuals, it does not illegally discriminate against transgender people" [1]. Reporting characterized it as the first federal appellate enforcement of restrictions on adult care [1]. The reasoning—that a procedure-targeted rule is not a person-targeted rule—is portable to any restriction framed around a class of treatment, and hormone therapy is such a class.

State-level data reporting is being proposed. Tennessee House Bill 754 would require clinics to submit patient data to the state including age, sex, medications, procedures, provider identity, and mental health diagnoses [15]. The re-identification objection is not speculative and has been stated plainly by community outlets: "If a report shows that in a small county, one 16-year-old began hormone therapy in March through a specific provider, it doesn't take a hacker to figure out who that might be" [15].

Coverage protections remain substantially intact in much of the country—the Movement Advancement Project's healthcare map, updated 25 August 2026, records that 54% of transgender people live in states where insurers are prohibited from excluding medically necessary transgender care, against 2% of LGBTQ people ages 13+ living in states that explicitly permit insurers to refuse it [7]. The picture is therefore not uniform collapse. It is sharp and worsening divergence, which for a national consumer product means the brand must be safe in the worst jurisdiction it ships to, not the median one.

4.3 Surveillance and health data

The Project On Government Oversight's investigation into enforcement technology enumerates the mechanisms: geofence warrants and cellular location data, data-broker purchases made without warrants, health data collection and sharing between providers, prescription records, and driver's license records—noting in one instance a request for more than 16,000 Texas driver's license gender-change records [10]. On applications specifically, it warns that "location data from apps that track our physical whereabouts, communications through popular apps like Facebook and Instagram, health data from apps that monitor and track our physical well-being, and browser search histories paint a detailed picture of our inner lives" [10]. That investigation is dated June 2023 and predates the 2026 developments in §4.2; it should be read as a floor.

The project's own business plan correctly establishes that HIPAA does not bind a direct-to-consumer application. The positioning consequence is the one the plan already draws—trust must be earned by disclosure rather than by claiming a regulatory badge—but there is a second consequence the plan does not draw, and it is a naming consequence:

> *Every artifact bearing the product's name is a potential disclosure. The App Store purchase record, the recurring line item on a bank or card statement, the notification banner, the home screen icon label, the receipt email, and the device backup filename are all strings the user does not control and cannot redact.*

A name that encodes "hormone," "trans," "T," or "HRT" converts each of those artifacts into an outing surface. This is the justification for the discretion test in §6.1, and it is the single strongest argument against retaining xT as a consumer-facing name—not that xT is ugly, but that its one piece of semantic content is the piece that should not be there.

4.4 The DIY population

A substantial fraction of the addressable population is not clinician-supervised. Prevalence estimates for do-it-yourself hormone therapy vary enormously by region and population: a 2022 review reports figures from 11% of transgender people in Ontario at the low end to 79% of trans women in Rio de Janeiro at the high end, with 31% in London, 49% among trans women in San Francisco, and 63% among trans people of color in San Francisco [3]. Reported

drivers include difficulty finding knowledgeable and non-stigmatizing clinicians, cost, exclusion, discomfort in clinical relationships, and a desire for faster transition; prevalence is generally higher among trans women, plausibly because testosterone's controlled-substance status makes transmasculine DIY harder [3].

For xT this cuts two ways, and the tension is worth stating rather than resolving.

It is where the product is most valuable. A user without regular bloodwork is exactly the user for whom a superposition model with a stated error band is the best available estimate of where their levels are. The PK paper frames the clinical gap as the interval between blood draws; for this population the interval is unbounded.

It is where the product is most dangerous. The same user has no clinician to catch the model when it is wrong, and xT's own current revision documents a defect that overstates endogenous testosterone by more than an order of magnitude at treatment onset for transmasculine users. A brand that positions as authoritative to an unsupervised population is making an implicit promise the model cannot yet keep.

The resolution is not to exclude these users but to let the brand carry the epistemic status honestly—which favors a name meaning *estimate*, *model*, or *instrument* over one meaning *truth*, *health*, or *care*. Names in the latter family—Verity, Attest, TrueLevel and their relatives—were generated and discarded on this ground.

4.5 The Euphoria case

The most instructive failure in this category is not a technical one. Euphoria.LGBT (later Solace) launched a suite of trans-focused applications with venture backing and prominent endorsement, and drew sustained community criticism. A 2022 critique published by the Massachusetts Transgender Political Coalition objected on grounds worth listing individually, because each is a naming and positioning lesson rather than a product one [9]:

1. **The tagline.** "Transition as a service" was read as commodifying transition—treating it as a purchasable product rather than a personal process.
2. **Linearity.** A checkbox-based, staged model of transition contradicted lived experience; the author writes that "no trans* person I know has had or wanted a linear transition."
3. **Unsourced authority.** Content was published with "no citations or author credentials."
4. **Exclusion.** No material specifically by or for trans people of color, disabled trans people, or low-income trans people.
5. **Extraction.** Fears of "data mining and invasions of LGBTQ+ privacy," and objection to the ethics of venture capital in this space specifically.
6. **Redundancy.** "All the information they share is already being circulated via online and in-person LGBTQ+ communities."

Three of these bear directly on xT's brand.

On the name itself. "Euphoria" names the emotional payoff of transition and sells it back. It is the archetype of the warm, community-legible name, and it is precisely the register that produced the commodification objection. This is direct evidence for the founder's instinct in rejecting the community-native branding axis: in this category, emotional naming reads as extractive when it comes from a commercial entity, and the warmth that is supposed to build trust does the opposite.

On sourcing. The "no citations or author credentials" objection is the one xT is structurally best positioned to answer, and it should be answered in the brand rather than in a footnote. A product that publishes its derivations is immune to the specific criticism that killed the best-funded entrant in its category.

On linearity. The critique of staged, checkbox transition models is a live risk for xT specifically, because the application ships a Stage System that binds named clinical scales—Tanner staging among them—to timeline copy. That feature is defensible as clinical information and indefensible as a prescription of how transition should proceed, and the difference is almost entirely a matter of naming and copy register. It is out of scope here but flagged: the brand should not imply a finish line.

5 Category positioning

5.1 The brief as a coordinate, not a checklist

The two selected axes—clinical credibility and discretion—define a plane. Placing the category's actual inhabitants on it is more informative than any of them individually.

	Low discretion	**High discretion**	
High clinical credibility	*Balance* (menopause; clinician-fronted, but unmistakably a menopause app)	**Largely vacant.** Regimen approaches it but is coded to male optimization by its descriptor and compound library.	The target region.
Low clinical credibility	*Euphoria/Solace*, *Transcapsule*, *Hormone Helper* — names that disclose the user	*Trans Memo*, generic medication reminders	Where the trans-specific segment actually sits.

Two structural observations follow.

The target region is vacant for a reason that is not inaccessibility. It is vacant because the two axes pull naming in opposite directions along the dimension of *specificity*. Clinical credibility is conventionally signalled by naming the domain— "Hormone Helper" is legible precisely because it says hormone. Discretion is achieved by not naming the domain. A name cannot resolve this by compromise; a half-disclosed name is simply a disclosed name. It resolves only by relocating the credibility signal off the noun and onto the *register* of the word—which is what an obscure technical term does, and what a warm word cannot.

This is the whole argument for a technical-register name. A term drawn from measurement, mathematics, or instrumentation carries clinical credibility through its diction while carrying zero disclosure through its denotation. A clinician reads competence; a bystander reads nothing. No other naming family satisfies both axes simultaneously.

5.2 Institutional weight over warmth

Section 3.3 established that the dominant competitor is a learned expectation of abandonment. Brands answer that expectation with signals of permanence, and the vocabulary of permanence is institutional: the name of a laboratory, an instrument, an observatory, a standard. It is not the vocabulary of companionship.

This aligns with, and independently supports, the founder's rejection of the community-native axis. It also reframes what "clinically credible" is buying. It is not primarily buying provider adoption or grant eligibility, useful as those are. It is buying the impression that this is a thing that will still exist in four years—which is the objection actually forming in a prospective user's mind, whether or not they articulate it.

5.3 App Store mechanics

The constraints are fixed and favorable to an abstract brand.

Field	Constraint and consequence
App name	30 characters; roughly the first 26 display in iPhone search results [8]. Holds "*Brand*: *Descriptor*" comfortably.
Subtitle	30 characters, displayed beneath the name [8]. A second keyword surface.
Keywords field	100 characters, not user-visible, indexed for search [8]. Carries "hrt, testosterone, estradiol, injection, trans" without any of those words appearing on the user's device.
Category	Health & Fitness is the correct category and triggers the declaration in §5.4. Hormone Helper's Lifestyle placement avoids that at the cost of search presence [6].

The keywords field deserves emphasis because it resolves the apparent tension between discretion and discoverability. Search terms live in a field the user never sees and that never appears on their device. A maximally discreet brand name loses nothing in ranking for "hormone tracker" provided the descriptor and keyword fields are used. The discretion tax on discoverability is close to zero; the discretion tax falls entirely on brand-term recall, which is a marketing cost, not a search cost.

5.4 The claim boundary: Apple's medical device declaration

This subsection sits outside the requested scope and is included because it constrains what the brand may assert.

As of 26 March 2026, Apple requires that an app be assigned a regulated-medical-device status in App Store Connect if its primary or secondary category is Health & Fitness or Medical, or if it is marked as containing frequent references to medical or treatment information in the age-rating questionnaire [2]. Apple defines regulated medical device apps as "those that function on their own or as part of a system for a range of medical purposes, including diagnosis, prevention, *monitoring*, and treatment of diseases and physiological conditions" (emphasis added). New apps must declare from that date; existing apps in the EEA, UK, and US must declare by early 2027, after which "you'll no longer be able to submit app updates" [2].

xT will be a new app in Health & Fitness and will therefore declare at submission. The declaration itself is routine—an app that is not a regulated device selects No. The positioning consequence is the constraint that comes with it: the marketing register and the declared status must agree. A brand and tagline built on *prediction of physiological state* sit closer to "monitoring" than a brand built on *logging and estimation* does. This argues against names and taglines in the diagnostic register—Assay, Verity, and anything reading as a measurement of the user rather than a model of a schedule—independently of their trademark profiles.

The safe formulation is the one the project has already written into its own disclaimer: xT estimates hormone levels from a pharmacokinetic model; it does not replace bloodwork or medical advice. The brand should be consistent with that sentence, which means the name should denote a *model* or a *curve*, not a *measurement* or a *verdict*.

5.5 The position to claim

Synthesizing §3.4, §3.3, §5.1, and §5.4:

> *A maintained, technically serious instrument for people managing hormone therapy—one that publishes how it works and where it is wrong, names nothing about its user, and is built to still be here in four years.*

Every clause is load-bearing. *Maintained* answers the graveyard. *Publishes how it works and where it is wrong* is the only differentiator that survives comparison with Regimen and the only one that answers the Euphoria critique. *Names nothing about its user* is the safety requirement. *Instrument* rather than *monitor* keeps the claim inside the declaration boundary.

Notably, the phrase "for trans people" does not appear—not because the product is not for trans people, which it emphatically is and should say everywhere except on the user's device, but because the brand layer is the one layer that has to travel onto a lock screen in a hostile state. Trans-specificity belongs in the App Store description, the landing page, the community posts, and the grant applications. It does not belong in the string.

6 Rebrand

6.1 The discretion test

"Discreet" is not a feeling a name gives; it is a property that can be tested. Four tests operationalize it, in rough order of how often each situation occurs.

Test	Question
Lock screen	A notification arrives while the phone is face-up on a desk, or is read over a shoulder on transit. Does the app name, plus a plausible notification body, disclose that the user is on hormone therapy? Note that the notification *copy* is as much a disclosure surface as the name—"Time for your injection" passes; "Estradiol dose due" does not.
Financial record	A recurring subscription line item appears on a bank or card statement, a shared family plan, or an expense report. Does the descriptor disclose? This surface is not fully controllable: the App Store receipt shows the app name as listed.
Over the shoulder	The home screen icon and its label are visible to a partner, parent, employer, roommate, or border officer. Does the icon disclose—by wordmark, by symbol, or *by palette*?
Compelled disclosure	A device is examined under legal compulsion or coercion. Nothing protects the user here except that the name and icon are not self-incriminating on their face, and that the vendor holds as little as possible. This test cannot be passed by naming alone; it is listed because it sets the ceiling on what naming can accomplish and argues for the data-minimization posture separately.

The third test has an implication the founder's own design language currently fails, and it is worth stating plainly because it is otherwise easy to miss in a naming document. The existing visual identity—accent `#E8709A` rose against `#7E74D8` lavender—is, in 2026, a legible trans signifier. A rose-and-lavender glass icon on a home screen discloses roughly as much as the word "hormone" would. Discretion is a property of the mark, not only of the word, and §6.5 treats it accordingly.

6.2 Screening method

Each candidate was screened on five criteria:

Discretion
Against the four tests above.

Clinical register
Whether the word's diction reads as technical rather than consumer, without asserting a diagnostic claim (§5.4).

App Store collision
Whether a storefront search for the bare word returns competing results that would bury the brand.

Trademark screen
United States federal records via Justia, filtered to classes 9, 42, and 44, with class 5 (pharmaceuticals) noted where relevant. *Indicative only; see §2.4.*

Sayability
Whether the word survives being said aloud to a clinician, read by someone who has never seen it, and typed from memory.

6.3 Screened candidates

1. Isocline — recommended

An isocline is a curve along which a quantity holds constant—in a differential system, the locus of points sharing the same slope.

Rationale	It names the mathematics of the product rather than its subject matter. The application's entire function is the shape of a curve over time, and an isocline is the object one draws when asking where a system is heading rather than where it is. It denotes a *model*, not a *measurement*, which keeps it on the correct side of the claim boundary in §5.4.
Discretion	Passes all four. Discloses nothing; reads as geology, mathematics, or a consulting firm.
Register	Strong. Unambiguously technical diction; carries credibility without a health noun. Works equally as a product name and as the name of a research entity ("Isocline Research"), which matters for the eventual nonprofit arm.
App Store	Clear. A storefront search surfaced no competing application.
Trademark	Clear in the relevant classes. Four marks located: Turin Golf (class 28, 2000), Huron Graffenstaden (class 7, 1991), Centre d'Etudes du Fraisage (class 7, 1982), and Isocline Ventures LLC (class 36, financial services, 2014). Nothing in classes 9, 42, 44, or 5. The three industrial and sporting marks are old enough that their live status should be confirmed but are in unrelated classes regardless.
Domain	Assume `isocline.com` is held by Isocline Ventures LLC. `isocline.app` and `isocline.health` are the realistic targets. **Unverified—check at a registrar.**
Weaknesses	Four syllables and a spelling that has to be heard twice. Semantically precise but not self-explanatory—the meaning has to be told, once, in the About screen. That is a real cost and it is the main argument for Ephemeris or a shorter name instead.

2. Ephemeris — principal alternate

An ephemeris is a table giving the computed future positions of a body at regular intervals—the oldest form of the thing xT is.

Rationale	The semantic fit is the best of any candidate considered. An ephemeris is precisely a precomputed trajectory: not a measurement of where something is, but a principled prediction of where it will be, tabulated against time, produced from a model and periodically corrected against observation. That is a one-word description of a superposition PK engine with blood-test calibration. It also carries genuine institutional gravity—ephemerides are published by observatories, and have been for centuries, which is exactly the durability signal §5.2 calls for.
Discretion	Passes all four.
Register	Strong, with one caveat: astrological ephemerides are the more common consumer encounter with the word, and that association cuts against clinical credibility for some readers while being neutral-to-charming for others.
App Store	Contested but not in health. "Ephemeris: Moon & Sun Tracker," "iPhemeris Astrology," and "The Photographer's Ephemeris" occupy the term. None is a health application, so category browsing is unaffected, but a bare brand-term search is crowded.
Trademark	Moderate. Approximately 30 records. Two in class 9: Moonshadow Mobile (2014) and The Photographer's Ephemeris / Crookneck Consulting (2015). Others in classes 33, 25, 14, 3. None in class 42 or 44. The class 9 marks are the concern and warrant an attorney's look.
Domain	**Unverified.** Assume the `.com` is gone.
Weaknesses	Long, hard to spell from hearing, and the astrology association is a genuine liability against the "clinically credible" half of the brief—the one place where this candidate is weaker than Isocline despite the better semantics.

3. Bateman — best clearance, disqualifying association

Harry Bateman, whose equation the application implements.

Rationale	Eponymous names carry more institutional weight than any other family: they read as a laboratory, a standard, or a firm, and they are the strongest available answer to the abandonment prior in §3.3. It is also the most honest name on this list — the product is a Bateman-function engine.
Discretion	Passes all four completely; a surname discloses nothing.
Trademark	The cleanest screen of any candidate. Only two records with the exact mark: Compass Group Holdings (class 43, catering, 1994) and Modern Fabrication Inc. (material handling, 2019). Nothing in classes 5, 9, 42, or 44.

Weaknesses	**Patrick Bateman.** The protagonist of *American Psycho* is, for the exact demographic this product serves—young, online, extremely fluent in reference—the first and possibly only association the name will produce. A trans health application named after a fictional misogynist serial killer is a headline, and the community documented in §4.5 is more than capable of writing it. This is not a reason to hesitate; it is a reason to decline. Listed because its clearance profile is genuinely the best found and the reasoning should be on the record.

4. Titer — clinical, with a pronunciation problem

A titer is the measured concentration of a substance in a solution.

Rationale	Short, real, clinical, ungendered, and directly on-subject without naming a hormone.
Discretion	Passes all four in writing.
Register	Strong but on the wrong side of the claim boundary. A titer is a *measurement*. xT does not measure; it estimates. A name asserting measurement sits closer to the "monitoring" language in Apple's device definition (§5.4) and over-claims against the product's own disclaimer.
App Store	Clear. No competing application surfaced.
Trademark	Crowded in the adjacent space. Roughly 90 records, 59 in class 9—but predominantly laboratory-apparatus compounds (TITERMAX/Promega, TITER ON CHIP/InDevR, TITERSEAL, TITERTUBE) rather than the bare mark. Dense enough in scientific class 9 to make clearance uncertain.
Weaknesses	**Said aloud, "titer" is ambiguous between *TIE-ter* and a first syllable that reads as "tit."** For a trans health application—and specifically for a transfeminine user saying the app's name to a clinician, a pharmacist, or a partner—this is not a minor infelicity. It is a name that can embarrass its user in exactly the interaction the product exists to support. Recommended against on this ground alone.

5. Interval — the plainest defensible option

The dosing interval; also the gap between blood draws that the product exists to fill.

Rationale	The naming rationale writes itself out of the project's own pharmacokinetic paper, which frames the clinical gap as the period *between* draws where patients have no visibility. Interval names that gap. It is also the easiest candidate to spell, say, and remember.
Discretion	Passes all four. Reads as a timer or a fitness app, which is close to perfect camouflage.
Register	Neutral. Plain rather than technical; carries less credibility than Isocline or Ephemeris.

App Store	**Poor.** "Interval" in a storefront search returns interval timers and HIIT applications in volume. Brand-term search would be unwinnable.
Trademark	No bare-mark conflict located in classes 9, 42, or 44—the field is dominated by interval-training marks in class 41 and by Interval International in travel. The generic-word problem is larger than the conflict problem: a bare common noun is weak as a mark and hard to enforce.
Weaknesses	Wins on plainness, loses on both halves of the brief — insufficiently distinctive to own, insufficiently technical to signal seriousness.

6. Plateau — rejected on semantics, not clearance

The steady-state plateau a repeated-dose regimen converges to.

Pharmacokinetically apt: reaching plateau is what a titrated regimen is for. Clearance is mediocre—284 records, including Plateau Systems in class 42 (software, 2001) and Menicon in class 9—but survivable.

It fails on meaning. In the vocabulary of transition, "plateau" does not mean *therapeutic steady state*; it means *stopped changing*, and it is a word users encounter in anxious forum posts about whether their development has stalled. Naming the application after the fear it is meant to allay is a fault no visual identity repairs. Retained in this list rather than in §6.4 because the clearance work was done and the semantic failure is the interesting part.

7. xT — the incumbent

The founder did not select "keep xT" as a brand posture. It is benchmarked here anyway, because a rebrand recommendation that has not beaten the incumbent on the record is not a recommendation. The case for retention is real and is stated at full strength.

In favor. It is two characters, which is a genuine asset—it fits any lockup, any icon, any 30-character App Store name with 27 characters left for the descriptor. It is already the name in the codebase, the repository, the running heads of two research papers, and the author's own usage; renaming has a real cost measured in documentation, continuity, and the small credibility that accrues to a name that has been used consistently. It is ungendered in the sense that mattered originally: it does not commit the product to estrogen or to testosterone. It has no plausible trademark conflict, because two-letter marks in unrelated classes rarely collide in practice. And short alphanumeric names age well in software.

Against. The decisive objection is not aesthetic. It is that xT's only semantic content is the wrong content. The "T" reads as testosterone—the founder's own business plan says exactly this, that "xT at least gestured at testosterone/hormones"—and that gesture is the single piece of information the discretion analysis in §4.3 says must not be on the device. A name that is otherwise opaque, but whose one legible element is the hormone, fails the lock-screen and over-the-shoulder tests specifically for the users in the worst jurisdictions.

Three further objections are weaker but real. It is unsearchable: a two-character string cannot rank or be recalled, and cannot be spoken to a friend as a recommendation, which matters disproportionately in a market whose only real acquisition channel is peer recommendation. It reads as unfinished, because it is—a codename signals a project, not an institution, which is the opposite of the durability signal §5.2 identifies as the category's central requirement. And it has no room for a research arm: "xT Research" does not read as an organization that publishes.

Verdict. Retention is defensible for a private beta and indefensible at public launch. If the name changes at all, it should change before the App Store listing, the domain, the

privacy policy, and the entity filing—all of which are imminent per the project's own 90-day plan, and all of which are more expensive to change afterward than the name is to change now.

6.4 Rejected candidates

Eleven names were generated and discarded. The reasons are more useful than the names, because each identifies a failure mode that will recur in any further naming work.

Name	Failure mode	Detail
Sextant	**Discretion, catastrophically**	Semantically excellent—an instrument for fixing your position when you cannot see land. The string begins with SEX. For a product whose brief is that the name must be safe on a lock screen and a bank statement, this is disqualifying on sight. Also crowded: 178 records including Sextant Medical in class 44.
Serum	Collision	Elegant camouflage—cosmetic to a layperson, clinical to a clinician. But the App Store holds several SERUM applications, and Xfer Records' Serum is a widely known audio product almost certainly registered in class 9.
Depot	Collision	A depot injection is exactly the route modeled. Home Depot makes the brand term unsearchable.
Almanac	Collision	Semantically strong (periodic quantities tabulated across time). "Almanac Health" already exists in the App Store—a direct collision in the exact category.
Assay	Trademark + claim	Clean in the App Store, but a live bare ASSAY mark exists in class 44 (medical services), owned by Assay LLC, alongside class 9 marks held by PerkinElmer and others. Independently, "assay" asserts measurement and sits on the wrong side of the claim boundary (§5.4).
Isochron	Trademark, in pharma	Better semantics than Isocline for a time-domain model. But five class 5 pharmaceutical registrations exist (Forest Laboratories, Sun Pharmaceutical), plus class 9 and 42 marks. Isochron is a drug brand name. Unusable for a health product.
Nadir	Valence	The trough of a dosing cycle. 169 records including a class 42 filing in February 2026. Independently, naming the product after the lowest point is the wrong promise.

Name	Failure mode	Detail
byTime	Collision (previously identified)	The project's prior rename candidate. Two existing iOS applications share the name, `bytimeapp.com` is taken, and the GitHub organization is claimed. The project's own monetization document already reached this conclusion; it is repeated here so the record is complete.
Euphoria-family	Register	Warm, emotional, transition-naming words. §4.5 documents what happened to the best-funded product that tried this. Excluded by the founder's brief and independently by the evidence.
Verity, Attest, TrueLevel	Over-claim	Names asserting truth or accuracy. The product's own published revision documents three defects including an order-of-magnitude baseline error. A brand may not promise what the model has disclosed it cannot deliver.
Half-Life	Trademark	Valve Corporation. No further analysis required.

6.5 Wordmark and visual direction

> **Superseded (October 2026).** The identity recommendations in this section — a grotesque lowercase wordmark, the "parallel slopes" mark, and the palette discussion — are retained as the record of this study but no longer describe the brand. The settled identity is the outlined ISOCLINE wordmark in IBM Plex Serif with the "I." small mark, rendered in ink, engraved hairline, or inkless emboss; see `docs/design/ISOCLINE_DESIGN_SYSTEM.html`. The discretion test argued for here is kept as a core brand principle.

Naming solves three of the four discretion tests. The over-the-shoulder test is solved by the mark, and the current identity does not solve it.

The palette problem

The existing design language—`#E8709A` rose and `#7E74D8` lavender on a near-black `#0A0814`, in glass cards—is well-executed and is the correct emotional register for the product's *interior*. As an *icon*, it is a disclosure. Rose-and-lavender gradients are, in 2026, among the most legible visual signifiers of trans-adjacent software, and a home screen is a public surface.

The recommendation is a split identity, which is a normal pattern and costs nothing:

Surface	Treatment
Icon and lock screen	Neutral. Monochrome or near-monochrome: graphite, slate, bone, or deep ink. A single geometric mark. No gradient, no rose. This is the only surface where discretion outranks warmth, and it should be designed as an instrument's badge rather than as an app icon.

Surface	Treatment
In-app	Keep the existing palette. Once the phone is unlocked and the app is open, the user has already chosen to be there. The rose and lavender belong here, and they are doing real work—the softness is what makes an honest, sometimes unwelcome data presentation bearable.
Landing page and store listing	Warm, and explicitly trans-first in copy. These surfaces are sought out, not encountered.
Research papers and entity	Austere. Serif, black on white, no accent color. The papers are the credibility asset; they should look like the literature they cite.

Typography

For Isocline specifically: a grotesque with a technical rather than a friendly cast—something in the register of a scientific instrument's faceplate. Avoid geometric humanists (the Poppins/Circular family), which read as consumer wellness and undercut the credibility the name is buying. Set the wordmark in lowercase or small caps with moderately open tracking; the word is long enough that tight tracking makes it unreadable at icon scale. Do not letterspace it into an acronym.

The mark

The obvious symbol is the curve itself, and it is worth resisting the obvious form. A literal plotted PK curve is a chart, not a mark, and becomes mush below 60 pixels. The extractable idea is that an isocline is a *family of parallel slopes*—a small field of short parallel strokes at a constant angle, which is both what the mathematics depicts and a shape that survives to 16 pixels. It reads as abstract and technical at every size and discloses nothing.

The landing page's existing background—a superposed multi-dose curve taken from the model's own output—should stay. It is genuinely good, and it belongs on the surface where the product is explaining itself rather than on the surface where it is sitting on a stranger's desk.

Naming lockup

Following the category convention (§5.3), with the 30-character limit in mind:

- App Store name: `Isocline: Hormone Tracker` (25 characters).
- Subtitle: something in the register of `Levels, doses, and lab results` (30 characters) — deliberately not naming a hormone, since the subtitle appears in the store listing but the *name* is what propagates to the device.
- Keywords field: `hrt, testosterone, estradiol, injection, trans, hormone, transition, taper` — invisible to the user, indexed by Apple, and present on no surface the user carries.
- Home screen label: `Isocline`.

Note the deliberate asymmetry: the store listing is findable and explicit, the device is silent. That asymmetry is the entire design.

6.6 Recommendation

Rank	Name	Basis
1	Isocline	Only candidate clear on all five screening criteria. Passes all four discretion tests; unambiguously technical register; no App Store collision; no located trademark conflict in classes 5, 9, 42, or 44; denotes a model rather than a measurement. Costs one sentence of explanation, once.
2	Ephemeris	Better semantics, worse everything else: crowded storefront, two class 9 marks, and an astrology association that works against the credibility half of the brief. Take it if the meaning matters more than the clearance.
3	xT (retain)	Defensible for private beta. Fails the discretion tests at public launch for the users who need them most.
—	Titer, Interval, Plateau, Bateman	Each fails one criterion decisively—pronunciation, distinctiveness, valence, and cultural association respectively. Documented rather than recommended.

Three actions follow, in this order, and the first two are cheap:

1. Verify `isocline.app` / `isocline.health` at a registrar and reserve the App Store name in App Store Connect. Both are same-afternoon tasks and both are prerequisites for the entity filing.
2. Have a trademark attorney run a real clearance search on Isocline in classes 9 and 44 before anything is printed or filed. The screen in this paper is not that search (§2.4).
3. Only then: entity name, privacy policy, landing page find-and-replace, and App Store listing. The landing page rename is, as the project's own monetization document notes, a find-and-replace.

7 Limitations

The trademark screens are not clearance. Stated in §2.4 and repeated here because it is the finding most likely to be over-read. Every "clear" in §6.3 means only that a Justia-indexed search of United States federal records surfaced no conflict in the named classes. State registrations, common-law marks, foreign filings, and likelihood-of-confusion analysis are all outside it.

Domain and App Store name availability were not established. No registrar or App Store Connect query was possible with the tooling available.

App Store figures are a single snapshot. Rating counts, prices, and last-updated dates were retrieved in August 2026 from storefront listings. Some competitor data—particularly for the abandoned applications—comes from a community-authored secondary source [14] rather than from listings, because the listings are gone or were not locatable. Those abandonment claims are consistent with the other evidence but were not independently verified listing-by-listing.

The positioning map in §5.1 is a judgment, not a measurement. No user research, survey, or interview informed this paper. Every claim about what trans users want is inferred from published community writing and from competitor outcomes. That is a real weakness: the single highest-value next step for this work is twenty conversations with prospective users, which would test the central claim of §3.3—that abandonment fear dominates feature preference—directly rather than by inference.

The medical and clinical axis is absent by scope. Provider adoption, WPATH and Endocrine Society positioning, and the software-as-a-medical-device question were depri-

oritized. Only the App Store declaration was carried in (§5.4), and only because it bounds the brand claim.

Two prominent sources are competitor marketing and are used for enumeration only (§2).

8 Conclusion

The question was whether to rename xT. The more useful finding was what the market turned out to be.

It is not a crowded field of HRT trackers that xT must out-feature. It is a segment whose trans-specific entrants are, with one exception, dead—Transcapsule since 2022, Solace since 2022, MyTransHealth since 2019—while the maintained, well-rated products adjacent to it are built for cisgender men optimizing testosterone and peptides. The opening is real. So is the reason for it: users in this category have learned that tools built for them get abandoned, and that prior is a more formidable competitor than any feature set.

The project's own business plan identified the pharmacokinetic model as the moat. It is not: Regimen ships a one-compartment Bateman superposition visualizer with cited sources and stated limitations. The asset that survives comparison is narrower and better—xT publishes its derivations, its out-of-sample validation, and, uniquely, its own defects. In a market defined by earned distrust, a published defect list outperforms a feature.

The name follows from that. It must signal durability rather than warmth, because durability is the doubt. It must signal technical seriousness through diction rather than through a health noun, because the health noun is a disclosure. It must denote a model rather than a measurement, because that is what the product is and what its own disclaimer says. And it must survive appearing on a lock screen, a bank statement, and a home screen in a state that is, as of March 2026, permitted to restrict adult care.

Isocline satisfies all of these and cleared every screen this study could run. Ephemeris is the better metaphor and the worse brand. xT remains adequate for a private beta and should not survive the App Store listing—not because it is a poor name, but because its one legible character names the hormone, and that is the one thing the string must not do.

The visual identity needs the same split the name does: austere on every surface the user carries in public, and warm everywhere the user has chosen to be. The rose and the lavender are not the problem. Their location is.

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